

ZENONCO.IO: SERVICE DELIVERY AND BRAND-BUILDING DILEMMAS

Jones Mathew, Sandeep Puri, and Manavi Chopra wrote this case solely to provide material for class discussion. The authors do not intend to illustrate either effective or ineffective handling of a managerial situation. The authors may have disguised certain names and other identifying information to protect confidentiality.

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Savitri Ahale was a low-income resident of Dharavi in Mumbai, India. She was unaware that she had been suffering from stage 4 colon cancer—a terminal diagnosis—until a check-up in June 2022 following excruciating bouts of stomach pain. Individuals in Ahale’s economic category, who fell below the poverty line (BPL), often prioritized their livelihoods over aches and pains, which could lead to ignoring issues until they became debilitating or affected their lives.¹ Unfortunately, as in the case of the millions who suffered in ignorance, cancer had afflicted Ahale long before she found herself in situation where treatment options were no longer available to her, and she died three months later. For patients in the BPL category, early detection of life-threatening diseases such as cancer had, and continued to be, a challenge.

Dimple Parmar, co-founder and chief executive officer (CEO) of ZenOnco.io (ZenOnco), launched the company with co-founder Kishan Shah, in November 2019, to provide end-to-end cancer-care guidance to different categories of customers. ZenOnco grew from an advisory and educational organization to an oncology health-care provider as its business model evolved from a not-for-profit unit to a for-profit business-model-driven firm. However, even after three years, ZenOnco struggled in the start-up stage with low brand awareness. In mid-July 2022, Parmar and Shah brainstormed over how to build ZenOnco into a strong brand in cancer care and improve service delivery. After discussions, Parmar and Shah laid forth a three-point agenda. First, popularize the brand and make it more widely recognizable and memorable. Second, make it the preferred destination for free scientific advice and counselling. Third, solve service delivery problems that were hindering its growth out of the start-up stage. They wondered what strategies would help them make the brand more memorable and get more BPL patients access to better cancer-care services and drugs at low cost.

THE IDEA

Parmar met Nitesh Prajapat in 2015 when they were both students at the same business school in India. Their entrepreneurial passion brought them together, so much so that when Prajapat was diagnosed with cancer in June 2016 and their independent entrepreneurial journeys stalled, Parmar had to choose between her women’s footwear start-up and Prajapat’s health—and his health won. Parmar studied everything she

¹ BPL (below poverty line) is an economic benchmark related to threshold income and is fixed by the Government of India. It was last determined by the Suresh Tendulkar Committee in 2011 and was fixed at ₹33 (US\$0.44) in urban areas and ₹27 (\$0.36) in rural areas per day for money spent on food, education, health, transport, and electricity; source: “BPL, Below Poverty Line,” Clear Tax, April 6, 2023, <https://cleartax.in/g/terms/bpl-below-poverty-line>.

could find on cancer treatment for Prajapat. The two juggled cancer treatment sessions, their start-ups, and college. Finally, after a year of treatment, they were ready to begin a new phase of life together. Parmar says of those days, “It was a difficult phase for both of us because in college one has dreams, ambitions, and a different set of to-dos: like graduating successfully, landing a dream job, or making the most of the school years. In our case, something else was destined.”

Between the end of 2016 and the beginning of 2018, Parmar and Prajapat saw his cancer metastasize to reach stage 4. With doctors giving a prognosis of only six months, they married. Shortly after, Prajapat passed away.

Turning Point

In trying to put her life back together, Parmar came to a decision. She wanted to dedicate herself to the cause of caring for cancer patients. She quit her banking job at the Bank of New York Mellon Corporation, saying, “I resigned the day Prajapat was admitted into ICU [intensive care unit]. I told my manager I wouldn’t be able to come back no matter the outcome because I had my inner calling, and I wanted to contribute to alleviating cancer. Millions of people are going through this journey, suffering, losing hope, and dying. If I could give hope and support to even a small number of those people, I would have a worthwhile life. That’s the mission that I took on.”

Doctor Michael Lerner: Guide and Formative Influence

As often happens, a chance meeting with somebody inspirational set the way forward. One such opportune interaction with Doctor Michael Lerner (Dr. Lerner), a pioneer in scientific health techniques, particularly cancer-related, motivated Parmar to work with cancer patients.² Dr. Lerner, as co-founder and president of Commonweal, worked in many areas of health and healing, including the study and alleviation of cancer, as well as being the author of *Choices in Healing* (offering healing choices based on scientific evidence) and creator of Beyond Conventional Cancer Therapies for the free dissemination of scientifically supported cancer-care information.³ Dr. Lerner became the formative influence for Parmar’s journey toward creating ZenOnco.

Love Heals Cancer

Based on Dr. Lerner’s work, Parmar developed a holistic understanding of the disease, the various therapeutic to pursue, and the best approach to follow for individual cases. She returned to India and started “Love Heals Cancer” (LHC), a non-profit venture to support cancer patients.⁴ LHC primarily focused on helping BPL patients, offering the essential basics for those affected by cancer: nutritious food, sanitation, hygiene, and nutraceutical supplements. LHC helped with the basic tools to deal with cancer that poorer patients struggled with daily. Simultaneously, Parmar explored the challenges hospitals, doctors, patients, survivors, caregivers, and the entire cancer-related system faced. Her conclusion: The sector was very fragmented and disorganized. She felt a tinge of uncertainty: Could anything significant be done in such a fractured industry, even with the help of advancements in science and technology?

² Dr. Lerner received a bachelor of arts from Harvard University in 1965 and a philosophy doctorate in political science from Yale University in 1971. He also received the MacArthur Prize Fellowship in 1984 for contributions to public health. He taught political science and psychology at Yale before moving to Bolinas, California in 1972. He co-founded Full Circle, a residential centre for children with learning and behaviour disorders in 1973.

³ Commonweal, founded in 1976, worked as centre for learning and research, transformation, and discovery. Its programs worked in the areas of health and healing, education and the arts, environment, and justice. One of the projects related to cancer. Over the past fifty years, Commonweal had run many programs around health and well-being, cancer, and diabetes for underprivileged individuals, especially children; Michael A. Lerner, *Choices in Healing: Integrating the Best of Conventional and Complementary Approaches to Cancer* (Cambridge, MA: MIT Press, 1996).

⁴ “Let’s Create a World Free from Cancer,” Love Heals Cancer, accessed July 7, 2022, <https://lovehealscancer.org/>.

Issues with the Love Heals Cancer Business Model

Soon after launching her LHC, Parmar realized that the non-profit model limited her vision for a more sustainable and scalable approach to managing cancer care on a larger scale. The non-profit model faced challenges. Many patients believed that non-profit firms should provide services free of charge. On the other hand, others did not value the services because they assumed they were of poor quality due to their low, nominal pricing. Parmar and her team struggled to develop a scalable and sustainable business model. There were also difficulties in providing personalized care and guidance, which were critical to cancer treatment. LHC lacked the necessary resources to provide high-quality, customized services. From a revenue perspective, LHC was run almost entirely on widely fluctuating donations and there was no stable source of funds. Some months, these fluctuations rendered even basic operational activities impossible. As a result, LHC was forced to focus on constantly raising funds rather than pursuing its core purpose of serving cancer patients.

THE ONCOLOGY SCENARIO IN INDIA

Parmar observed a significant difference between the United States and India in a cancer patient's diagnosis-to-recovery experience. The entire US medical support group, including the oncologist, district nurse, practitioner, financial aid person, and social support team, would be with a patient to care for their every requirement. The United States had the world's highest age-standardized cancer incidence rates (362.2),⁵ which was in contrast to India, with the lowest age-standardized cancer incidence rate worldwide (97.1).⁶ Yet, around 70 per cent of patients in India were diagnosed in advanced stages. The primary obstacles to early cancer diagnosis and treatment in India had been the unavailability of proper knowledge, misleading information, lack of trust in public health systems, and unequal health expenditure across different population segments.

Given India's size and the highly varied nature of medical access and awareness levels, the struggle for scientific information, easy affordability, and lack of support continued at each step. Doctors, even in large hospitals, expressed concerns about working in silos without a support team. Further, cancer patients often struggled to find a doctor for general health problems. In 2018, the doctor-patient ratio in India was about 1 to 2,000, and the requirements for oncologists and dedicated cancer centres were predicted to be at least 5,000 and 550, respectively, compared to the existing 1,250 and 250. Due to this shortage, oncologists were severely constrained in dealing with an affected population of 70 million and found it difficult to provide appropriate and timely support to their patients. As a result, India had one of the highest cancer mortality rates in the world, which was four to six times higher than in the United States.⁷ The low oncologist to patient ratio also meant that oncologists in India could not spend sufficient time with patients and caregivers. Overstretched resources were always a problem.

Parmar stated, "Cancer is a disease that does not have a single-point solution. A patient has to find out what choices are available, accessible, and affordable and whether those choices are backed by scientific facts to decide the therapies or the treatment modalities to be pursued."

⁵ Age-standardized figures are a weighted average of the age-specific rates per 100,000 persons.

⁶ Age-standardized figures were weighted to take into consideration the differences between the number of people across different age groups in the populations of different countries; Zia Sherrell, "What to Know about Cancer Survival, Incidence, and Death Rates," *Medical News Today*, July 28, 2021, <https://www.medicalnewstoday.com/articles/cancer-rates-by-country>.

⁷ "Big C May Affect 70M Indians by 2021," *The Hindu Business Line*, January 12, 2018, <https://www.thehindubusinessline.com/news/variety/big-c-may-affect-70m-indians-by-2021/article9527233.ece>.

THE BIRTH AND GROWTH OF ZENONCO

In cancer care, one of the biggest concerns was the severe lack of time for the patient to wait and experiment with different forms of treatments. Timely action in the right direction was of paramount importance. In addition, there were different challenges with the disease in different patient segments. A BPL patient needed basic food and sanitation; middle-class patients needed to find a cure and a chance to improve their quality of life. Wealthy patients were ready to pay any amount to get the best treatment. Parmar wanted to serve all of them. She brushed aside professional advice that suggested she should be more specific in targeting specific market segments: “I want to serve all segments. My dream is big. Helping a small number of patients is not my wish.”

It was at this point that Shah joined Parmar. Shah, too, was a student from Parmar’s business college cohort. As a qualified chartered accountant and chartered financial analyst, Shah brought financial acumen to the business. Both Parmar and Shah shared the vision of positively impacting the lives of many cancer patients and filling the cancer-care gap. Together, they started ZenOnco in November 2019 with the vision “to save [the] lives of cancer patients” and the mission “to make integrative oncology accessible to all and extend the life of cancer patients.”

Parmar stated, “The values we stand for are high integrity, compassion, empathy, and quality patient care and experience.”

The ZenOnco System

Parmar developed the ZenOnco system around six core elements: 1) patient counsellors; 2) collaborations with external service facilitators; 3) technology applications; 4) support community; 5) doctors, oncologists, and hospitals; and 6) diagnostic laboratories, pharmaceutical companies, and retail pharmacies (see Exhibit 1). Each element was designed to deliver a high-quality, compassionate patient and caregiver experience enhanced by a supportive CANNECT environment. However, there were issues of service failures: improper matching of patients with the right service provider; inability of the partner service provider to offer the desired service at the right time due to technical issues or resource shortfalls; patient counsellors not having the right levels of experience or training in handling patient concerns; feedback failures resulting in lack of proper improvements in the service process; and non-completion of the service process by a patient or caregiver.

Parmar developed a set of six success metrics for customer experience and satisfaction (see Exhibit 2). At ZenOnco, a patient went from being received by the counsellor to being educated, guided, emotionally supported, connected with the appropriate external service facilitator, and then assured by follow-up services to fulfill their cancer treatment journey (see Exhibit 3).

ZenOnco’s Service and Positioning Change

In the medical domain, confusion existed between alternative and integrative medicine. “Alternative” meant choosing between options and pursuing only one school of medicine. “Integrative” meant combining more than one treatment protocol. Other treatment protocols complemented allopathic (i.e., conventional) medical treatment, so it became integrative. ZenOnco was clear in deciding the nature of its service offering—it was to be an “integrative oncology” (leading to the “io” suffix to its name). Parmar emphasized, “We were very clear from the beginning that we would have the words ‘integrative oncology’ in our brand name and offer purely scientific facts and evidence as part of our approach.”

The positioning was clear: ZenOnco would be for all people with cancer, without segmenting the population. Anybody afflicted by the disease was qualified to be a ZenOnco customer.

In the medical fraternity, ZenOnco's credibility was entirely dependent on the scientific validity of the information provided on its platform through licensing agreements with research agencies. Doctors were more open to conversations with patients about complementary treatment protocols. As long as complementary approaches did not interfere with the main medical treatment, ZenOnco found greater acceptability of integrative oncology approaches by medical professionals. Over time, ZenOnco started receiving patients from mainstream oncologists who believed that its integrative oncology approach had benefited their patients in the past. But it was still more of a trickle than a flow. More needed to be done to increase the number of referrals.

Initially, ZenOnco worked alone and faced many challenges in serving patients. It began to create tie-ups with multiple institutions to better serve its patients. The network of these collaborations gradually grew to more than two hundred super-specialized cancer hospitals and more than a thousand oncologists across India. However, the high dropout rate from treatment due to the social stigma attached to the disease and the high treatment cost—monetary, physical, and emotional—continued to be a problem. Further, despite many outreach programs by various hospitals and non-governmental organizations, awareness, and knowledge levels of the importance of screening remained very low. In India, over 70 per cent of patients were diagnosed too late, when recovery probability decreased significantly, and costs increased substantially. Further, accessibility was limited due to a shortage of qualified specialists. ZenOnco had to work on rapidly acquiring patients and expanding its oncologist and hospital networks.

Branding ZenOnco

Health care, particularly cancer care, was a very sensitive field with a highly emotional aspect to it, especially if someone had experienced it personally. Therefore, Parmar and Shah were very cautious in how they built the brand because what they were doing was completely new in India's integrative cancer care. Making people understand the concept of integrative oncology was also a challenge because it was unfamiliar and was perceived differently by different people. From a branding perspective, ZenOnco had changed its approach and focused more on offering what patients were searching for: information and tips that might help them during treatment.

The name ZenOnco was a modern-sounding portmanteau of Zen philosophy and oncology. The ZenOnco logo was an aesthetically pleasing crossed ribbon in pastel shades of green and blue (see Exhibit 3). Parmar felt that brand building in the health-care domain could be based on more than just a sales approach. In cancer care, brand building depended on guiding, educating, and caring for patients. And yet, despite adopting a patient-centric approach from its inception, the brand needed much greater market visibility.

Parmar and Shah also tried to determine whether cancer survivors could be inspired to be brand ambassadors, since they might be willing to help and give back to society. Could they be made the face of ZenOnco in their cities and communities? Parmar and Shah were confident that this would be a win-win situation for both—survivors would get a chance to fulfill their desire to help others living with cancer, and ZenOnco would be able to reach out to more people by training the survivors and making them brand ambassadors and even counsellors. Brand awareness and brand recall had to be revved up.

Entrepreneurship Style—Prioritize and Do It Now

In the initial phase, ZenOnco offered complementary care services focusing on healthy diet, emotional and physical wellness, survivor connections, and other support services. But shortly after, the COVID-19 pandemic hit, and ZenOnco started receiving general medical-related queries and online appointment requests.

In May 2020, ZenOnco started adding medical support services to its offering. It also began to collaborate with hospitals, doctors, oncologists, diagnostic laboratories, pharmaceutical companies, and pharmacists in the system, which made it truly integrative. Parmar's experience as a cancer caregiver gave her close-range knowledge of the issues and mistakes people made. ZenOnco followed a continuous feedback system, wherein the company talked to patients and better understood their experiences. Instead of waiting for the perfect time or idea to make improvements, ZenOnco adopted an agile approach, like a start-up entrepreneur.

ZenOnco continuously added services to its portfolio when it saw rising demand for a particular service. Cancer patients faced hundreds of problems throughout their journey, and it was impossible to cater to all of them. ZenOnco tried to identify the "moments of truth," the most critical ones, which could be easily scaled up. It projected the needed resources, expenses to be incurred, and revenue that could be generated for every new service. After that, it decided on the appropriate timing to launch each service.

This approach had been working well for ZenOnco. The ability to deliver services backed by technology helped to reduce response time. For ZenOnco, there were three key ingredients in everything that it undertook: a scientific approach, technological support, and the offer of hope.

Ziopar

Parmar was innovating at a good speed. Soon after LHC and ZenOnco, the entrepreneurs launched Ziopar, the world's first artificial intelligence (AI)-based tool for preliminary integrative oncology assessment report generation. The value proposition of this innovative product was that with a one-time effort, by offering six information points in an online form—covering cancer type and affected organs, cancer stage, prior treatment taken, performance status, symptoms, and side effects, and other medical co-morbidities (if any)—a patient could receive a detailed report on suggested medical treatment, diagnostic tests, side effects, and symptoms management based on international guidelines and protocols, free of cost. This kind of personalized AI-generated advice was designed to help patients and caregivers educate and empower themselves to make their own choices and ask the right questions of their doctor.

Ziopar was recognized and approved by the European Society for Medical Oncology (ESMO) in September 2020. By 2020, Ziopar had helped over 21,000 patients in sixty-three countries. Shah said, "Being recognized globally at the ESMO Virtual Congress 2020, where we presented an e-poster titled, 'Artificial Intelligence-based tool to generate directional integrative treatment reports for cancer patients,' on September 17, 2020, and the response received for it was testimony to our dedication to making quality integrative cancer care accessible to all."⁸

In-house Patient Counselling Support

ZenOnco developed an in-house tool to help counsellors guide patients and their families in the appropriate direction for treatment. The final decision remained in the doctor's hands, and counsellors connected patients with the right experts. There were four steps to this process. The first step was to make the patient aware of trustworthy and reliable educational and scientific information and knowledge about the choices available to them once they were diagnosed with cancer. The second step was to help them discover the right expert for a particular service. The third step was to fulfill their validation (i.e., ensuring that the patient partook of the suggested service). The fourth step was to receive and give continuous feedback, monitor the process, and ensure that the patient was able to follow the prescribed regimen.

A key element in the entire process was community support. Parmar stated, "We believe that in health care, especially cancer care, good quality community support provides answers to frequently asked questions."

⁸ K. Shah and D. Parmar, "ESMO Virtual Congress 2020," OncologyPRO, accessed February 24, 2023, <https://oncologypro.esmo.org/content/search?searchText=ziopar>.

CANNECT

In June 2021, ZenOnco launched the CANNECT mobile application (app) for cancer patients, caregivers, survivors, and experts to connect with other patients with a similar cancer type. The app aimed to facilitate community sharing and learning.

The challenge was that most users were already part of existing online groups on social media platforms such as Facebook, Instagram, Telegram, and WhatsApp, and it was difficult to get these users onto CANNECT. Nevertheless, Parmar was contemplating making CANNECT a preferred app for cancer patients and their caregivers by weaning them away from the social media community sites they were already on, which often had fake, misleading, and unverified information.

Further, Parmar wanted to develop CANNECT as a companion tool (and not just as a one-time doctor's appointment booking app) in cancer-related areas like education, self-help tools, getting a cancer coach, doctors' opinions, and connecting with other survivors and patients. CANNECT would match the medical and geographical profiles of users and connect them to a community network. The app worked on the premise that a patient or caregiver would want to connect with another patient or caregiver with similarities in their disease journeys and location. Community spaces in the health-care sector were not only scarce in number but also lacked well-moderated, verified information sites for community members. CANNECT filled this gap.

CANCER CARE: A COMPLEX SPACE

According to Parmar, cancer was a vast field where, at every stage, multiple variables had to be investigated. When a patient arrived for treatment or advice, many parameters had to be analyzed—the current stage of cancer, the type of cancer, the patient's mutations, their past treatment protocols, the side effects they were experiencing, the type of drugs they were on, any co-morbidities, their current activity level, and their city of residence. Based on these ten factors, the Ziopar tool gave recommendations regarding the type of oncologist to consult and the treatment needed by the patient. Following this stage, the company arranged a visit to the appropriate specialist and informed the patient of the treatment course to be pursued.

Similarly, more than 100 scientifically backed complementary and alternative medicines were available in the country to manage side effects. However, like in the treatment of other diseases, customization was a key factor.

Pricing of Cancer Medicines

Cancer medications made up a major portion of the treatment burden. Parmar said, "One of the most expensive spending heads would be the cost of medicines. For example, the cost of medicines needed for a lung and bone cancer treatment costed around ₹0.5 million⁹ to ₹0.6 million out of a total treatment cost of approximately ₹12 million to ₹15 million. Cost variations would depend on factors such as the type of cancer, cancer stage, hospital type, geographic[al] location, etc."

Most high-priority cancer medicines identified by Indian oncologists were generic chemotherapy agents that provided substantial improvements in survival chances. However, access to these medicines was limited due to the significant financial burden patients experienced, especially uninsured patients. This was particularly acute within the private health system. According to a recently published article:

⁹ ₹ = INR = Indian rupee; ₹79.20 = US\$1.00 on July 7, 2022; all currency amounts are in ₹ unless otherwise specified.

Doxorubicin, cyclophosphamide, paclitaxel, carboplatin, methotrexate, cytarabine, capecitabine, fluorouracil, and gemcitabine were the popular drugs that oncologists in India prescribed. These drugs were not easily accessible, and the financial burden was “catastrophic” for almost all income groups but severely so for the most economically vulnerable populations. The risk of major out-of-pocket expenditures for each medicine was reported by 19–58 per cent of oncologists. The risk of crippling expenditure was reported by 58–67 per cent of oncologists for cytotoxic drugs like Rituximab and Trastuzumab. The risks of financial toxicity were substantially higher within the private health system than the public system.¹⁰

The National List of Essential Medicines (NLEM) in India was established in 1996 and had since undergone periodic revisions. The inclusion of a drug in NLEM ensured price control under the Drugs (Price Control) Order and therefore improved affordability and access.¹¹

In May 2019, the National Pharmaceutical Pricing Authority brought nine anti-cancer drugs under its price control purview. It cut prices by as much as 87 per cent to reduce the burden on cancer patients and their families. For example, the price for a Pemetrexed 500 milligram (mg) injection was reduced from ₹22,000 to ₹2,800; the 100 mg price from ₹7,700 to ₹800; for a pack of ten tablets of Erlotinib 100 mg, the price was cut from ₹6,600 to ₹1,800; and for the 150 mg pack from ₹8,800 to ₹2,400.¹²

THE ONCOLOGY MARKET IN INDIA

India allocated a meagre share of public expenditure (around 1.5 per cent of its gross domestic product) to health care. This was a cause of the increasing occurrence of various incurable illnesses due to late or no diagnosis and poor knowledge of such diseases, especially in rural areas. In addition, a substantial shortage of trained medical and health personnel and a lack of facilities made access to good treatment highly challenging. Sedentary lifestyles coupled with unhealthy eating habits, along with alcohol and tobacco abuse, led to various types of illnesses, including several types of cancer.¹³

Enhanced technology complemented a large volume of ongoing research to offer affordable cancer treatment. Several new technologies were notable. Epigenetic treatment helped transform cancerous cells into healthy ones with much less toxicity than chemotherapy. Cloud-based new-generation sequencing systems aimed to create diverse gene pools (worth around \$16.87 million in 2020 and growing at a rapid pace). Cutting-edge technology—such as cyclotron and positron emission tomography—computed tomography (PET–CT), nanotechnology, regenerative gene therapy and liquid biopsies, and new-generation photodynamic cancer therapy, to name a few—had been introduced on the Indian subcontinent.¹⁴

The growing trend of cancer patients was alarming. Data showed that India reported 26.7 million cancer patients in 2021, a figure that was projected to increase to 29.8 million by 2025.¹⁵ This explosion in cancer incidence called for extensive investment in prevention techniques, vaccinations, cancer awareness, and

¹⁰ Maju Sengar, Adam Fundytus, Wilma Hopman, C. S. Pramesh, et al., “Cancer Medicines: What Is Essential and Affordable in India?,” *JCO Global Oncology* no. 8 (2022) e2200060, DOI: 10.1200/GO.22.00060.

¹¹ “Drug Price Control Order,” Government of India, Department of Pharmaceuticals, accessed July 25, 2022, <https://pharmaceuticals.gov.in/dpconppa>.

¹² Pushpa Narayan, “Almost 90% Slash in Prices of Nine Anti-cancer Drugs,” *Times of India*, May 19, 2019, <https://timesofindia.indiatimes.com/india/almost-90-slash-in-prices-of-nine-anti-cancer-drugs/articleshow/69392319.cms>.

¹³ “Trends in Indian Cancer Market,” CyteCare Cancer Hospitals, accessed on August 2, 2022, <https://cytecure.com/media/trends-in-indian-cancer-market/>.

¹⁴ “India Oncology NGS Market Size, Share & Trends Analysis Report by End-Use (Clinical Research, Hospitals & Clinics), by Technology, by Application, by Product & Service, by Workflow, and Segment Forecasts, 2021–2028,” Grand View Research, accessed August 5, 2022, <https://www.grandviewresearch.com/industry-analysis/india-oncology-next-generation-sequencing-market>.

¹⁵ Priyanka Sharma, “India’s Cancer Burden to Rise to 29.8 Million in 2025: ICMR Report,” *Mint*, May 13, 2022, <https://www.livemint.com/science/health/indias-cancer-burden-to-rise-to-29-8-million-in-2025-icmr-report-11652382169284.html>.

screening. In addition, there was a need to increase focus on clinical data research and studies for creating better cancer-fighting techniques.¹⁶

The oncology market in India was estimated to grow by \$734.18 million between 2020 and 2025 at a compound annual growth rate of 13 per cent. India was said to have a highly complex cancer-care system. One of the main problems facing India was providing affordable and accessible cancer care. The COVID-19 pandemic helped move the whole system online with rapid digitization, which made it possible for patients from Tier 2 and Tier 3 cities to reach out for care.¹⁷ Reputable hospitals in the country implemented non-clinical therapies to support the overall well-being of cancer patients and their caregivers. Keeping prices affordable was a huge challenge, as was improving care penetration into more areas. According to Samara Mahindra, founder and CEO of CARER, there was huge scope for the adoption of integrative therapies across India's cancer sector at the national, regional, and local levels.¹⁸

The Main Competitors

Not many health-care technology start-ups existed in the integrative oncology field. However, new AI- and algorithm-driven entities such as Onco.com, Hospido, and DCode Care were present in the cancer-care market. The oncology care sector was booming due to its underserved nature and untapped potential in areas ranging from screening and diagnosis to palliative care, integrated treatments, psychological counselling, and digitized access.

Onco.com: Moving the Cancer-Care Needle

Onco.com, a Bengaluru-based start-up founded in 2017, aimed to be a one-stop destination for cancer care.¹⁹ It had recruited over 1,500 oncologists, had a network of over 500 treatment centres, and operated in eighteen countries, including India. It was funded by Accel, Chiratae Ventures, and development innovation global venture funds.²⁰

Hospido: AI Chatbot-Driven Cancer Teleconsultation

Hospido was founded in 2020.²¹ An innovative teleconsultation platform for those seeking cancer treatment, Hospido developed a completely automated chatbot called "Cancer Dost" (cancer friend) to help patients easily access cancer-related advice and tips.²² By January 2021, Hospido had more than a hundred leading oncologists on board, most of whom had trained at top-tier hospitals in India and the United States. In 2022, Hospido had established chemotherapy centres across north and central India, including Amritsar, Srinagar, Gwalior, Agra, and Kanpur.²³

¹⁶ CyteCare Cancer Hospitals, "Trends in Indian Cancer Market."

¹⁷ The Reserve Bank of India classified Indian cities into tiers based on 2001 population census numbers: Tier 1 = 100,000 people and above; Tier 2 = 50,000–99,999; and Tier 3 = 20,000–49,999; Reserve Bank of India, Annexure 5: "Details of Tier-Wise-Classification of Centres Based on Population," in Master Circular on Branch Authorisation, July 1, 2011, <https://www.rbi.org.in/Scripts/NotificationUser.aspx?Id=9817#an4>.

¹⁸ Prabhat Prakash, "Nation Struggling with Rising Cancer Burden, Majority Cases Are Diagnosed in Advanced Stages," ET Health World, July 5, 2022, <https://health.economictimes.indiatimes.com/news/industry/nation-struggling-with-rising-cancer-burden-majority-cases-are-diagnosed-in-advanced-stages/92679001>.

¹⁹ Sutrishna Ghosh and Shivani Muthanna, "Here's how Onco.com Is Gearing Up to Be a One-Stop Destination for Cancer Treatment," Your Story, February 19, 2020, https://yourstory.com/2020/02/upclose-startup-bengaluru-oncocom-cancer-treatment-care/amp?utm_pageloadtype=scroll.

²⁰ "Our Mission," Onco, accessed July 7, 2022, <https://onco.com/about-us>.

²¹ "Hospido," Your Story, accessed July 7, 2022, <https://yourstory.com/companies/hospido/amp>.

²² "Hospido Introduces AI Enabled Chatbot Cancer Dost," BioSpectrum India, accessed July 7, 2022, <https://www.biospectrumindia.com/news/98/17909/hospido-in-introduces-ai-enabled-chatbot-cancer-dost.html>.

²³ "Hospido.in Launches AI Enabled Chatbot Named Cancer Dost to Provide Information on Cancer," Pharmabiz, January 11, 2021, <http://www.pharmabiz.com/PrintArticle.aspx?aid=134711&sid=2>.

DCode Care: Optimizing Cancer Treatments and Prices

Founded in 2019, DCode Care was a discovery platform for patients to find high-quality cancer treatment options at the best prices.²⁴ Its investors and partners included Koox, Inflection Point Ventures, PointOne Capital, and ACCESS Health International. DCode Care offered special discounts for consultation fees (37.5 per cent discount), diagnostics (30 per cent discount), and treatment (20 per cent discount).²⁵ DCode Care also had a seamless patient journey flowchart to give visibility to patients of their prospective journey and how DCode Care could help them. It had access to 200 clinical resources required for cancer treatment as per complying with international standards.²⁶

By July 2022, nearly ten to twelve start-ups had come up in the oncology domain. They were driven less by pure profit motives and more by the urge to prevent India from becoming the world's cancer centre. Some founders, like Parmar, had started their ventures following a direct personal loss to cancer. Others wanted to create a system to overcome the woefully inadequate access and affordability issues for cancer patients in addition to early diagnosis and digitized access, irrespective of location. Whether profit or altruism was the driver, the number of companies had increased in the sector that ZenOnco was trying to make a mark in.

ZenOnco was like these start-ups with respect to four characteristics: the digitization of the access and delivery platform; an initially small but growing scale of operations; non-profit to start but turning toward a for-profit business model; and founder-driven rather than lead by independent professionals. Similarities aside, ZenOnco differed from its competitors in one key way: It had the unique combination of its founder's personal loss as its motivation and her professional business management qualifications and experience.

THE WAY FORWARD

Parmar and Shah were looking to build brand equity for ZenOnco among cancer patients across the country, improving the service areas on their patient journeys, and scaling up to become a more impactful enterprise over the next two years. With more than a dozen start-ups vying for visibility and impact in the cancer-care sector offering consultation, treatment guidance, psychological support, access to oncologists, and drugs at heavily discounted prices, ZenOnco had to think of strategies that would allow it to ride the expected growth wave in the oncology market and to survive the stiff competition and make integrative cancer care accessible across all levels of society. Parmar and Shah needed to determine how to make ZenOnco a more widely known and identifiable brand that provided affordable and high-quality cancer care and community support to patients and caregivers.

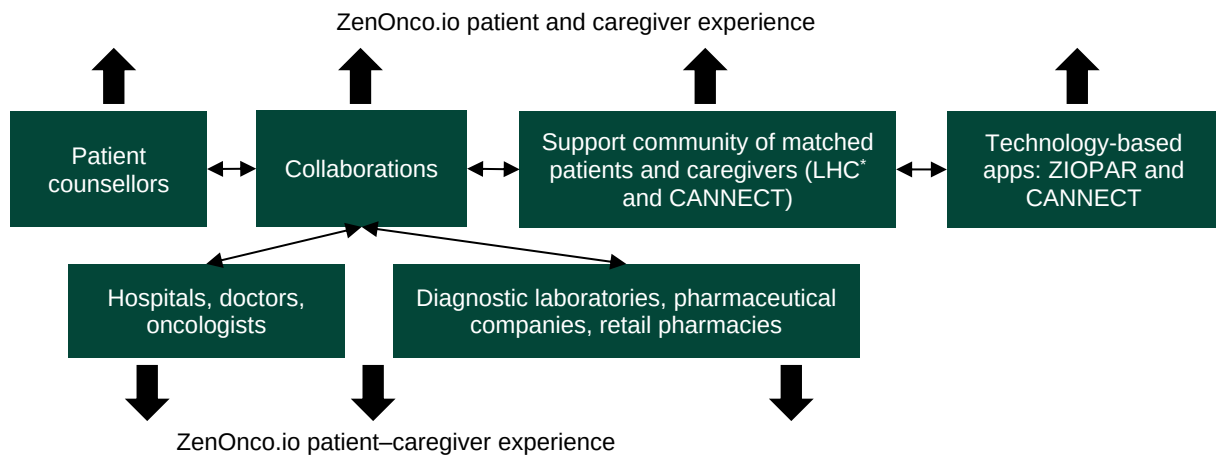
Three specific questions arose regarding brand positioning. Should the brand image be built on science? Would an emotional approach from the people's point of view be a better option? Or was the integrative approach the best way forward? Whichever positioning the team chose, the need to accelerate brand awareness and brand recall was urgent.

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²⁴ "DCode Care," Inflection Point Ventures, accessed July 7, 2022, <https://ipventures.in/portfolio/dcode-care>.

²⁵ "About Us," DCode Care, accessed July 7, 2022, <https://www.dcodecare.com/about-us>.

²⁶ "DCode Care Raises Seed Round Co-led by Inflection Point Ventures and PointOne Capital," Express Computer, August 16, 2021, <https://www.expresscomputer.in/news/dcode-care-raises-seed-round-co-led-by-inflection-point-ventures-and-pointone-capital/78421/>.

EXHIBIT 1: THE ZENONCO PATIENT AND CAREGIVER EXPERIENCE SYSTEM

Note: LHC = Love Heals Cancer.

Source: Created by the case authors based on company documents.

EXHIBIT 2: ZENONCO'S KEY METRICS

Area	Metric	Measurement
Patient benefit	Clinical outcome	Questionnaire on quality of life
Patient satisfaction	NPS	Percentage of promoters versus percentage of detractors
Service acceptance	Conversion rate	First-time paid patients in month/total new patients in month
Patient referral	Referral rate	Referral patients/paid patients over last 3 months
	Referral percentage	Referral patients/organic patients
Patient retention	Retention rate	Number of paid patients in current month from previous months/number of total paid patients in current month
Patient discovery	Organic leads percentage	Organic patients/total patients
	Website visits	Number of organic visitors (MoM percentage increase)

Note: NPS = Net Promoter Score; MoM = Month-on-Month.

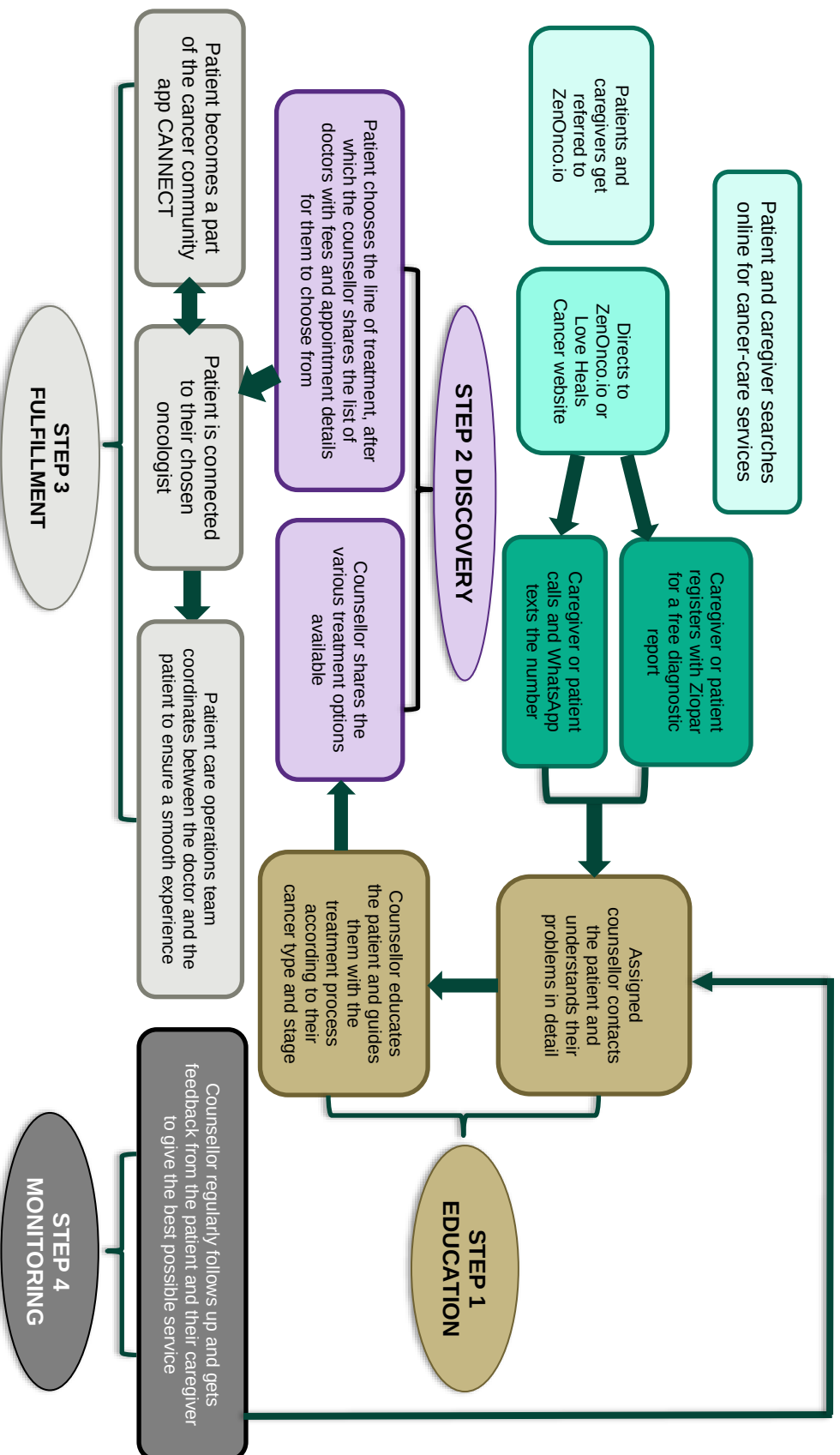
Source: Created by the case authors based on company documents.

EXHIBIT 3: ZENONCO LOGO

Source: Company documents.

EXHIBIT 4: THE CANCER PATIENT CUSTOMER JOURNEY KEY POINTS

Incorporation of feedback



Note: app = mobile application.
Source: Created by the case authors based on company documents.